

Treatment Algorithm: Urinary Tract Infection (UTI) in Children >2 Months Old

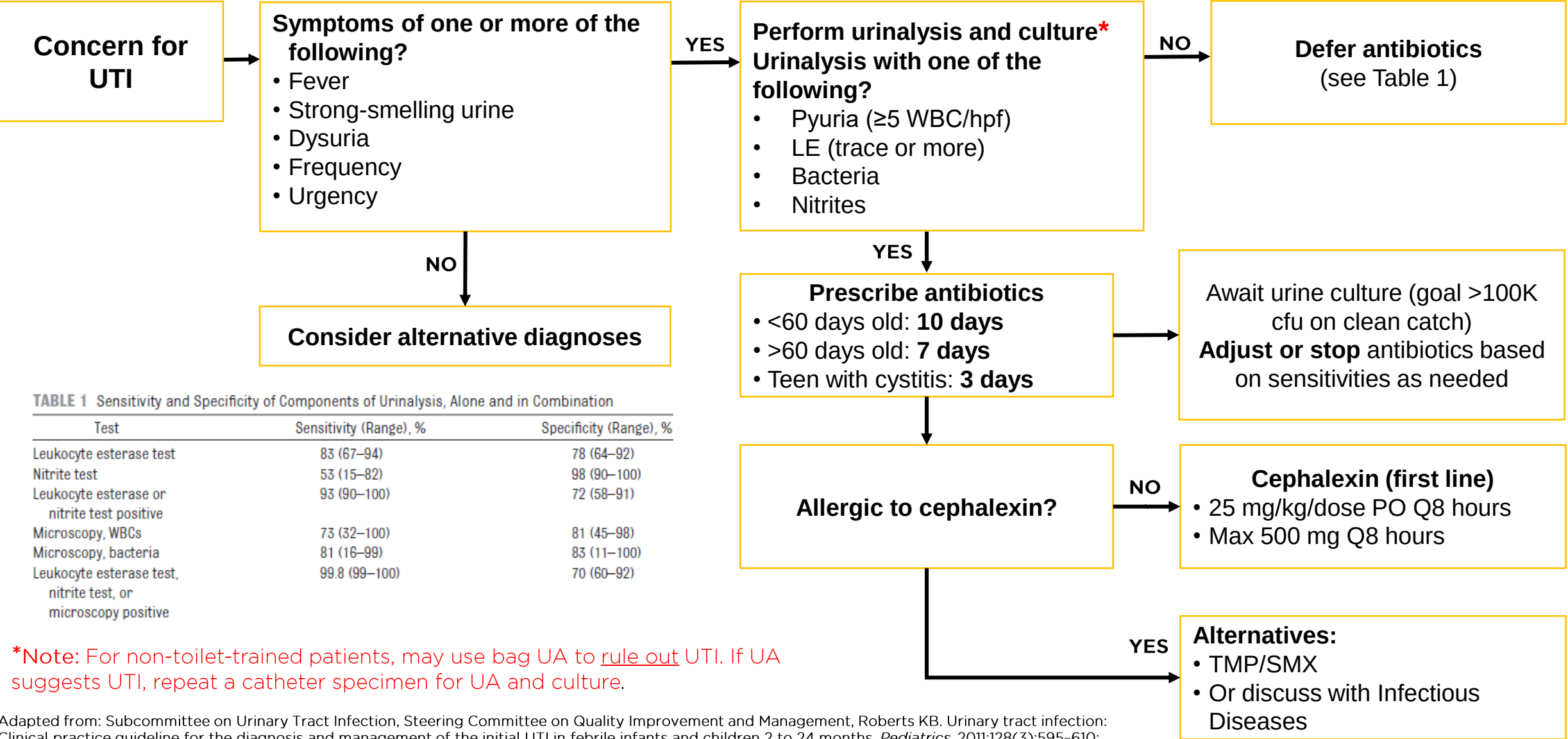


TABLE 1 Sensitivity and Specificity of Components of Urinalysis, Alone and in Combination

Test	Sensitivity (Range), %	Specificity (Range), %
Leukocyte esterase test	83 (67–94)	78 (64–92)
Nitrite test	53 (15–82)	98 (90–100)
Leukocyte esterase or nitrite test positive	93 (90–100)	72 (58–91)
Microscopy, WBCs	73 (32–100)	81 (45–98)
Microscopy, bacteria	81 (16–99)	83 (11–100)
Leukocyte esterase test, nitrite test, or microscopy positive	99.8 (99–100)	70 (60–92)

*Note: For non-toilet-trained patients, may use bag UA to rule out UTI. If UA suggests UTI, repeat a catheter specimen for UA and culture.

Adapted from: Subcommittee on Urinary Tract Infection, Steering Committee on Quality Improvement and Management, Roberts KB. Urinary tract infection: Clinical practice guideline for the diagnosis and management of the initial UTI in febrile infants and children 2 to 24 months. *Pediatrics*. 2011;128(3):595–610; SCH Clinical Standard Work Pathway.